

WHOM TO SCREEN?

INDIVIDUALS WITH FOLLOWING RISK FACTORS

PRESENT PROFILE



DIABETES MELLITUS



HYPERTENSION



CHRONIC KIDNEY DISEASE



HIGH LIPIDS



BMI (KG/M²) > 23



WAIST CIRCUMFERENCE
≥ 90 IN MEN, ≥ 80 IN WOMEN



SEDENTARY LIFESTYLE

PERSONAL HISTORY



CARDIOVASCULAR DISEASE
(STROKE, TRANSIENT ISCHEMIC ATTACK,
CORONARY ARTERY DISEASE)

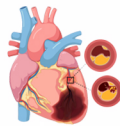


SMOKING & TOBACCO USE

FAMILY HISTORY



SUDDEN CARDIAC DEATH



CORONARY ARTERY DISEASE (CAD)
(Stent, Heart attack, Bypass surgery)

ISCHEMIC HEART DISEASE MAY CAUSE ANY OF THE FOLLOWING:

- STABLE ISCHEMIC HEART DISEASE
- ACUTE CORONARY SYNDROME
- HEART FAILURE
- SUDDEN DEATH
- ARRHYTHMIA

Disclaimer: This leaflet is currently being piloted and may contain inconsistencies. Please help us improve it by reporting any issues to KHDC Nepal.



DRUG LIST AND DOSE

Aspirin*	75-300mg
Clopidogrel*	75-300 mg
Atorvastatin*	20-80 mg
Metoprolol succinate*	12.5-50 mg
Isosorbide dinitrate*	5 mg

*National list of essential medicines, 6th revision, MOHP Nepal



NON-PHARMACOLOGICAL ADVICE



Regular Physical Activity



Reduce Stress



Low Salt Consumption



Quit Alcohol & Tobacco



Consume Healthy Diet



Weight Reduction



Adequate Sleep

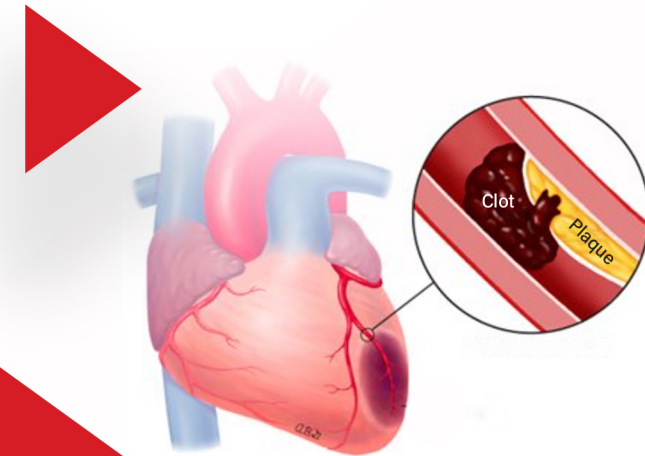
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ISCHEMIC HEART DISEASE IDENTIFICATION AND MANAGEMENT FOR PRIMARY CARE PROVIDERS

USE THIS GUIDE TO

- Identify patients with Ischemic Heart Disease (IHD)
- Manage and refer patients with IHD

Diminished blood flow through a coronary artery in the heart results in ischemic heart disease (IHD).

STABLE ISCHEMIC HEART DISEASE



CENTRAL ILL-LOCALIZED CHEST PAIN

Pain typically comes on with exertion, emotion or exercise (when heart rate increases)
It disappears with rest

ECG: Normal
TROPONIN*: Normal
PERFORM IF AVAILABLE

SUSPECT ANGINA

TO REDUCE PAIN: Isosorbide dinitrate twice daily
TO STABILIZE THE PLAQUE: Give atorvastatin 20 mg once daily
TO REDUCE HEART RATE: Give Metoprolol succinate sustained release 12.5 mg (If heart rate more than 75 beats per minute) once daily
FOR ANTI-PLATELET ACTION: Tab ecosprin 75 mg po orally after meal

REFER



Normal Artery



Fatty Plaque Buildup



Narrowed Artery



Ruptured Plaque & Clot

ACUTE CORONARY SYNDROME

CENTRAL ILL LOCALIZED CHEST PAIN

Lasting more than 20 minutes Occurring at rest or Increasing in frequency and severity
Pain will not be relieved after giving
Isosorbide dinitrate 5mg sublingual 3 times every 5 minutes

PERFORM ECG WITHIN 10 MINUTES

ECG: Abnormal
TROPONIN: Elevated*

*May be normal within 8 hours of chest pain or in cases of unstable angina

SUSPECT MYOCARDIAL INFARCTION

Aspirin 300 mg to be chewed and swallowed
Clopidogrel 300 mg stat
Atorvastatin 80 mg stat
ABSOLUTE BED REST

REFER

NOTE: SEEK TELEMEDICINE HELP FOR INTERPRETATION OF ECG



ABNORMAL ECG

New ST elevation at the J point in two contiguous leads of $>0.1\text{mV}$
In leads V2-V3: $\geq 1.5\text{ mm}$
Arrhythmias(VT/VF)

Note: If confusion with ECG, consult with telemedicine

CRITERIA FOR REFERRAL

- All cases of acute coronary syndrome after initial management
- Newly diagnosed case of stable ischemic heart disease
- Hemodynamic instability (hypotension, bradycardia or tachycardia)
- Dyspnea, swelling of legs or palpitation
- Shock, Heart Failure, Syncope

1/3RD DEATHS WORLDWIDE ARE
CAUSED BY A
CARDIOVASCULAR DISEASE.



CONTACT US

For any clinical queries / If anything in the document is difficult to understand



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